



● PARTNERSHIP PROPOSAL · VOLUME V OF V · MASTER DECK

A US revenue-cycle partnership for **Leverage Edu's** three growth surfaces.

Halcyon Medical Billing operates the full revenue cycle for US independent medical practices. We're proposing a single partnership umbrella with **three concurrent integration points** — Leverage's alumni community, Leverage Careers' employer accounts, and Leverage Edu's diaspora marketing channel. One agreement. Three monetisable surfaces. Each with its own KPIs and own sunset clause.

\$14.9B

US MEDICAL BILLING MARKET

155K+

LEVERAGE ALUMNI REACH

120+/yr

TNSDC NURSING PIPELINE

25% / 10%

YR-1 / RECURRING CHANNEL CUT

WHY THIS. WHY NOW.

Three reasons we wrote this proposal in May 2026.

SIGNAL · 01**TNSDC nursing pipeline is live.**

Leverage Careers' MoU with the Tamil Nadu Skill Development Corporation is now training 120+ nurses/year for global placement, starting in Vellore. Many will land inside US independent practices — the exact buyers Halcyon serves. Halcyon is the natural billing-services bundle on top of that placement conversation.

SIGNAL · 02**EBITDA-positive · IPO bankers tapped.**

₹375 Cr in FY26 (+112% YoY), EBITDA-positive, ₹2,000–3,000 Cr IPO in 12–18 months (Inc42, Apr 2026). Bankers will pay a premium for **recurring, non-counselling B2B revenue** — which is exactly the line a Halcyon partnership creates.

SIGNAL · 03**The affiliate engine already exists.**

Leverage already runs a partner dashboard with UTM tracking, monthly payouts, and the 1-1-1 commission guarantee. Adding Halcyon as a second economic flow on top of that infrastructure is configuration — not a build. Time-to-launch: 3 weeks.

The strategic frame: we are not asking Leverage to build a new product. We are asking to monetise three relationships Leverage *already* has — alumni, US employers, and the Indian-American physician community — with our economics layered on top. Halcyon brings the operations, the compliance, the technology, and the founders' direct lines. Leverage brings the audience.

WHAT HALCYON IS

A HIPAA-compliant revenue cycle for **independent** US medical practices.

US doctors lose 10–15% of earned revenue to denied claims, slow payments, and broken billing. Halcyon manages the entire cycle — eligibility, coding, claims, denials, payment posting, patient collections — on a real-time client portal. Field-level PHI encryption. Append-only audit trail. The standard hospitals operate on, delivered to a 1–10-provider clinic that can't afford to build it themselves.

COMPLIANCE

Registered LLP · BAA stack

Halcyon Medical Billing LLP. HIPAA + HITECH compliant. BAAs executed with every vendor in our stack. Lawyer-reviewed MSA template.

OPERATING MODEL

12-step RCM workflow · 60–90D results

Practices typically see AR days drop and denial rates fall within the first two billing cycles. We deliver measurable outcomes inside 90 days.

BUYER FIT

1–10 provider clinics · 6 specialties

Primary care, internal medicine, cardiology, psychiatry, orthopedics, family medicine. Identical to the buyer profile Leverage Careers serves.

96%

TARGET FIRST-PASS ACCEPTANCE

23%

RISE IN US INSURANCE DENIALS,
2024

\$44B

MARKET SIZE BY 2034 (PROJECTED)

72%

DEALS WON THROUGH REFERRALS

THE PROPOSAL AT A GLANCE

One umbrella agreement. Three concurrent surfaces.

SURFACE	WHAT LEVERAGE PROVIDES	WHAT HALCYON PROVIDES	OWNER AT LEVERAGE	YEAR-1 TARGET
A · Alumni Engine	Opt-in channel inside partner dashboard for 200–500 alumni in US healthcare	Halcyon CRM, training, scripts, all customer-facing operations	Head of Partnerships	20 closed practices
B · Employer Bundle	One-line handoff at end of every Leverage Careers employer call	20-min demo, contract, full onboarding within 30 days of intro	CBO / COO Platform	15 closed practices
C · Diaspora Channel	Co-branded landing page, quarterly mailers to Indian-American physician segment	100% creative production, \$58K Year-1 marketing co-funding	CMO	10 closed practices

Each surface ships independently. Each has its own KPI, its own go/sunset gate, and its own internal owner. Leverage can run all three, two, or one — we are agnostic. The umbrella agreement is structured so any single surface can be sunset without affecting the others. *Pages 5–7 detail each surface; Page 8 models the combined economics.*

SURFACE A · ALUMNI ENGINE

Your alumni already work where our buyers are.

Leverage has placed 155K+ students cumulatively, with a measurable cohort now embedded inside US independent practices as RNs, NPs, residents, and partners. Halcyon's highest-converting channel is warm introduction — **72%** of US medical billing contracts are won through referrals. A nurse who has worked in a clinic for two years is the exact warm intro we'd otherwise pay \$300–800 to generate cold.

The proposal: **open a Halcyon track in the existing Leverage partner dashboard.** Alumni who have landed in the US opt in, send a one-line introduction to their practice owner, and earn ongoing commission on top of their salary — with Leverage Edu earning a co-channel cut.

Why it converts

- Warm intros from internal staff close at **18–25%** vs ~2% cold
- The relationship trust lives with the alumnus — competitor cannot poach
- Recurring 10% monthly — the alumnus has incentive to keep practice happy
- Zero marginal cost to Leverage; opt-in only; no PHI flows

Per-practice unit economics

- Mid-range practice: **\$8,775 Year 1** + \$7,800 recurring/yr
- Small primary care: \$5,063 Year 1 + \$4,500 recurring/yr
- What Leverage books = (closes) × per-practice rev. No projection of close count.

SURFACE B · EMPLOYER BUNDLE

One employer call. Two revenue lines.

Leverage Careers is in active dialogue with US independent practices every week — selling them on hiring an H1B-sponsored RN or NP. Those clinics are 1–10 providers, billing insurance, often understaffed on admin. **They are identical to Halcyon's ICP.**

Today: one sale per employer call — the placement fee. Tomorrow: at the close of every employer call, the rep adds one line — *"Quick heads-up — we partner with a billing company called Halcyon. Want a 20-min intro?"* That single sentence converts at materially higher rates than any cold channel either company can buy.

Two integration depths

- **Light:** standard three-way intro email at close of placement — no CRM changes, no new fields
- **Deep:** co-branded landing page, UTM-tagged outbound, channel inside partner dashboard. 2-week build.

Per-practice unit economics

- Mid-range practice: **\$8,775 Year 1** + \$7,800 recurring/yr
- Channel scales linearly with placement volume — not a fixed projection
- No exclusivity, no clawback after Month 1, no upfront from Leverage

Compliance frame: Leverage is *not* a Business Associate under HIPAA in this model. No patient data flows through Leverage's stack. The introduction ends at the practice owner's name and email; Halcyon's BAA covers everything downstream.

Co-marketing into Indian-American physicians.

~125,000 Indian-American physicians practice in the US, AAPI is the largest ethnic medical society in the country, and they overwhelmingly own and run independent practices. Many of them are first-gen immigrants with kids in the Leverage funnel today — or who used Leverage themselves a decade ago. **The brand is already trusted in their home.**

Cold outreach into this group closes at 4–6%. With "vetted by Leverage Edu" attached, we model 18–25%. The conversion gap pays for the entire co-marketing budget — with Halcyon eating 100% of the marketing spend.

12-month plan

- Q3 '26: Co-branded landing page + email blast to physician parents segment
- Q4 '26: Joint webinar — "Running a US practice in 2026"
- Q1 '27: AAPI national conference co-sponsorship (\$25K, joint booth)
- Q2 '27: Quarterly featured editorial in Indian-American newsletter

Per-practice unit economics

- Halcyon spend: ~\$58K Yr-1 hard cost + 100% creative — Halcyon's risk, not Leverage's
- Mid-range practice: **\$8,775 Year 1** + \$7,800 recurring/yr
- Net commission to Leverage scales 1:1 with closes; no projection committed

THE HONEST MATH

Per practice that signs & stays.

Halcyon doesn't pay channel partners a salary, retainer, or upfront fee. The economics are 100% performance-based — they kick in only once a referred US practice signs and pays its first invoice. **25%** of the practice's first-month Halcyon fee, paid once. **10%** of every subsequent monthly fee, recurring as long as the practice stays. No cap. No expiry. Per-practice math below at INR/USD ₹94 (May 2026 spot).

PRACTICE SIZE	HALCYON FEE / MO	MONTH -1	RECURRING / MO	YEAR 1 / PRACTICE	YEAR 2 / PRACTICE
Small primary care \$75K/mo collections	\$3,750	\$938 (~₹88K)	\$375 (~₹35K)	\$5,063 (~₹4.76L)	\$4,500 (~₹4.23L)
Mid-range \$130K/mo collections	\$6,500	\$1,625 (~₹1.53L)	\$650 (~₹61K)	\$8,775 (~₹8.25L)	\$7,800 (~₹7.33L)
Multi-specialty \$250K/mo collections	\$12,500	\$3,125 (~₹2.94L)	\$1,250 (~₹1.18L)	\$16,875 (~₹15.86L)	\$15,000 (~₹14.10L)

What Leverage actually books = (practices that sign across all three surfaces) × (per-practice rev above). We are not projecting a number of closes. The number above is what's structurally guaranteed once a practice signs and stays. Recurring revenue, ~95% gross margin, zero CAC on Leverage's side — the highest-quality revenue line they could add for IPO disclosure.

What's a "close"? A US practice that signs a Halcyon MSA *and* has paid its first monthly invoice — at which point Halcyon issues the channel-commission ACH to Leverage. No commission on demos, MOUs, or unsigned proposals. **Halcyon eats 100% of integration, marketing, and sales-execution cost across all three surfaces.** INR/USD ₹94 (May 2026 spot).

WHAT WE'RE ASKING

A 30-min joint scoping call. Then a 90-day pilot.

We'd like one 30-minute call with whichever of Akshay, Rachit, Sandeep, the CMO, and the Head of Partnerships is right for the room. Halcyon brings Naitik (CEO), Rushan (CFO), and a one-page integration spec for each of the three surfaces. Decision out of the call — yes/no on a 90-day pilot, with the surface chosen by Leverage.

PHASE	DURATION	WHAT HALCYON SHIPS	WHAT LEVERAGE PROVIDES	DECISION GATE
Scoping	1 call · 30 min	Specs for A/B/C surfaces, MSA + Referral Agreement template	Right people on the call	Pilot scope agreed
Pilot	90 days	Integration build, training, comms, all attribution reporting	Audience access · 1 internal owner	Day-60 KPI review
Scale	Open-ended	All three surfaces live · CSM dedicated to Leverage	Continued audience access	Quarterly review
Sunset	30 days notice	Clean handover · final attribution report	30-day notice via email	No clawback owed

REPLY WITH AVAILABILITY BEFORE 27 MAY 2026 · NAITIK@HALCYONMEDBILLING.COM

THE HALCYON TEAM

Three founders. One direct line.

CEO & CTO · SENDER

Naitik Joshi · SENDER

Founder. Architected Halcyon's HIPAA portal, encryption layer, and the 12-step RCM workflow every client runs on. Will personally lead all technical demos.

COO · OPERATIONS

Ankit Chauhan

Owns billing operations & client onboarding. Handles every signed practice's first 90 days personally to protect the channel's retention KPIs.

CFO · SENDER

Rushan Khan

Owns commercial & partnership economics. Direct counterpart for Leverage's Partnerships, BD, CFO, and CMO functions on this proposal. Reply to naitik@halcyonmedbilling.com.

COMPLIANCE & COMMITMENTS

- Halcyon Medical Billing LLP — registered, audited, in good standing
- HIPAA + HITECH technical safeguards in place
- Leverage is not a BA under HIPAA — no PHI flows through Leverage's stack
- 30-day sunset clause in the umbrella agreement — either party
- Quarterly attestation reports for IPO disclosure-grade auditing

IF THE ANSWER IS YES

- Referral & Co-Marketing Umbrella Agreement — 4 pages, DocuSign in 48 hrs
- Integration spec for chosen surface(s) — live in 3 weeks
- Halcyon-funded creative & tooling — we eat the build cost
- Dedicated CSM at Halcyon for the Leverage channel only

NJ

Naitik Joshi

CO-FOUNDER, CEO & CTO · HALCYON MEDICAL BILLING LLP

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